

Supportive Care Considerations

◇ Neurologic Monitoring

- NIHSS at baseline and serially
- Neuro exam q15 min for 2 hours post-thrombolytic, q30 min for 6 hours, then hourly

◇ Glucose Control

- Target **140–180 mg/dL**
- Treat hypoglycemia promptly
- Avoid glucose >180 (associated with poor outcomes)

◇ Temperature Management

- Maintain normothermia ($\leq 37.5^{\circ}\text{C}$)
- Treat fever aggressively with antipyretics

◇ Oxygenation

- Supplemental O_2 if $\text{SpO}_2 < 94\%$
- Avoid hyperoxia unless indicated

◇ Swallow Evaluation

- Screen for dysphagia before oral intake
- Use NPO status until cleared

◇ DVT Prophylaxis

- Sequential compression devices (SCDs) for all
- LMWH or low-dose UFH after 24h if no bleeding risk

◇ Head Position

- Head of bed at 30° to optimize venous drainage unless contraindicated

References

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2. Jauch EC, Saver JL, Adams HP Jr, et al. Guidelines for the Early Management of Adults With Ischemic Stroke. *Stroke*. 2013;44(3):870–947. doi:10.1161/STR.0b013e318284056a
3. Qureshi AI, Ezzeddine MA, Nasar A, et al. Prevalence of elevated blood pressure in acute stroke. *Arch Intern Med*. 2007;167(8):866–872. doi:10.1001/archinte.167.8.866
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